



Difficult Encounters

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Special thank you to Dr. Sue St. Pierre
for her contribution to these slides

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Objectives

- Describe the characteristics and consequences of a difficult patient interaction
- ◊ Explain how physician, patient and situational factors contribute to a challenging interaction.
- ◊ List some general strategies for approaching difficult encounters
- ◊ Prepare for the SP Difficult Encounters lab on 10/30!

Reflective Exercise #1

Please spend the next 2-3 minutes pondering these two questions:

- (1) What patient characteristics or behaviors will be the most challenging for you?
- (2) What insights do you have about yourself that will help inform how you approach patients that you find challenging?

What do you envision will MOST make for a difficult encounter for you in your future practice?

Nobody has responded yet.

Hang tight! Responses are coming in.

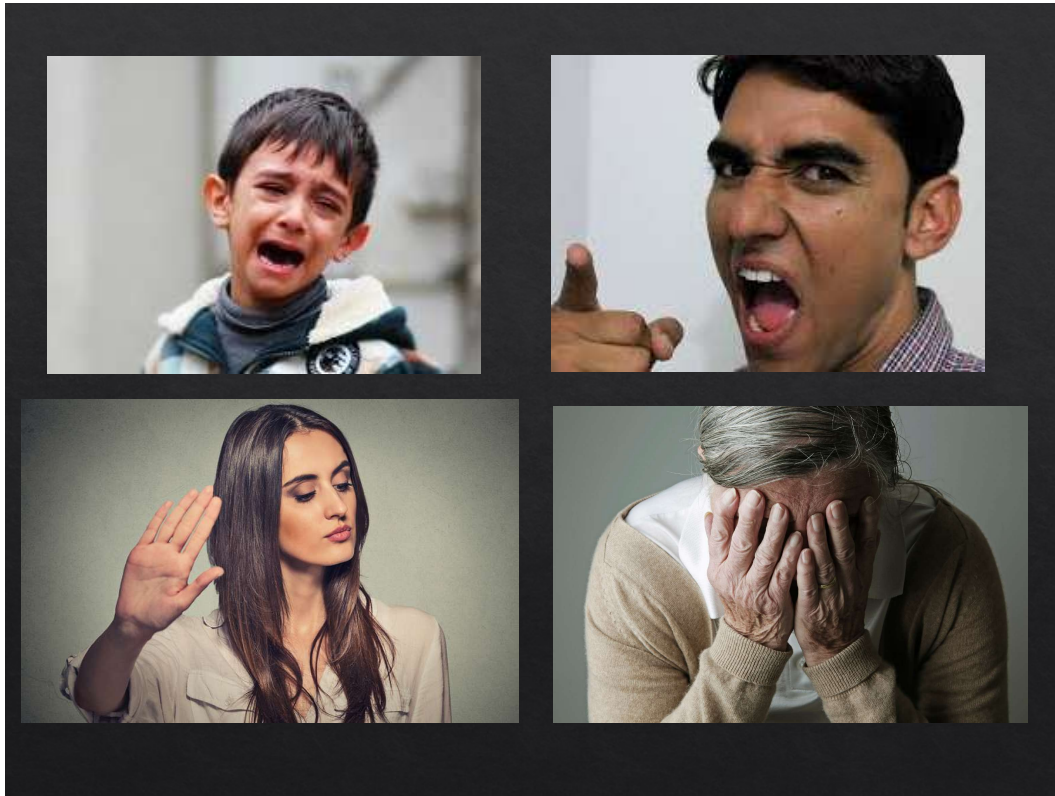
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More info at polleverywhere.com/support

What do you envision will MOST make for a difficult encounter for you in your future practice?

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Characteristics of Difficult Encounters

- ◇ Mismatch of expectations
- ◇ Unhappy patient and/or unhappy doctor
- ◇ Exhausting
- ◇ “Heartsink” or dread for the encounter

Physician Factors

Physician Factors That Can Lead to Difficult Clinical Encounters

Table 1. Physician Factors That Can Lead to Difficult Clinical Encounters

Attitudes	Conditions	Knowledge
Emotional burnout	Anxiety/depression	Inadequate training in psychosocial medicine
Insecurity	Exhaustion/overworked	Limited knowledge of the patient's health condition
Intolerance of diagnostic uncertainty	Personal health issues	Skills
Negative bias toward specific health conditions	Situational stressors	Difficulty expressing empathy
Perceived time pressure	Sleep deprivation	Easily frustrated
		Poor communication skills

Cannarella Lorenzetti R, Jacques CH, Donovan C, Cottrell S, Buck J.

Managing difficult encounters: understanding physician, patient, and situational factors. Am Fam Physician. 2013 Mar 15;87(6):419-25. PMID: 23547575.

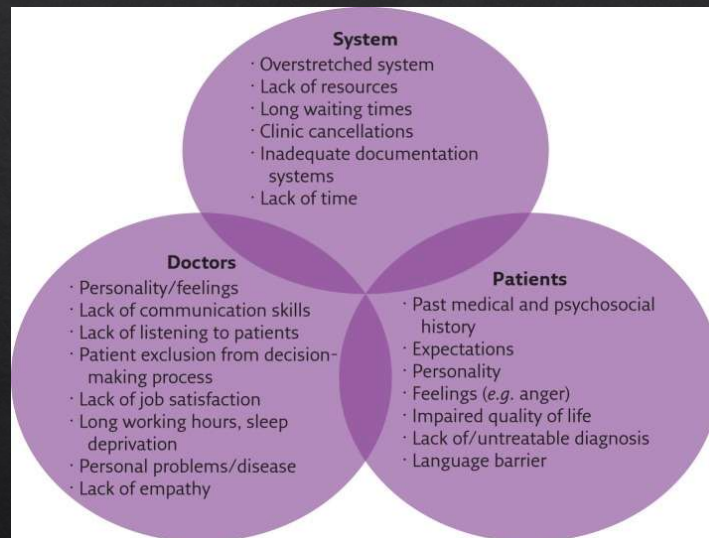
Patient Factors

- ❖ Behavioral issues
 - Angry/argumentative
 - Highly anxious or hypervigilant to body sensations
 - Reluctance to take responsibility for own health
 - Demanding / entitled behavior
- ❖ Worry/fear about cause of symptoms
- ❖ Chronic pain syndromes, substance use disorders, somatic disorders
- ❖ History of physical or emotional abuse
- ❖ Financial constraints or insurance issues causing difficulty with adherence to therapy recommendations
- ❖ Low health literacy
- ❖ Psychiatric diagnoses (personality or mood disorders)
- ❖ Multiple concerns needing to be addressed, some more medically urgent than patient's chief complaint

Situational Factors

- ◆ Constraint of a typical 15-minute office visit or patient census load inpatient
- ◆ Low reimbursement/pressure to see more patients
- ◆ Availability of medical knowledge (Dr. Google) such that patients potentially come in with more questions, expectations or concerns
- ◆ Availability of outside test results (23&Me, Modern Fertility, private MRIs, etc.) with unclear evidence / interpretation
- ◆ Patient surveys tied to physician bonuses
- ◆ Staffing shortages (Administrative, medical assistants, nursing and provider)
- ◆ Technological challenges or barriers for virtual appointments

Components of a Difficult Encounter



<https://pmc.ncbi.nlm.nih.gov/articles/PMC5467659/>

Reflective Exercise #2

- ◇ It is 4:55 on a Friday afternoon. Your last patient is Pat Smith, a 54-year-old with high blood pressure and diabetes. Pat arrived 7 minutes late today for a 15-minute appointment and has a list of 6 things to discuss with you. His blood sugar log was left at home and your MA tells you Pat's blood pressure is 195/105. He says he didn't take his BP meds yet today because he was busy this morning and forgot. You need to leave clinic at 5:15 as your son has a state championship baseball game.
- ◇ What is most frustrating about this scenario?
- ◇ How do you think do you would deal with it?

What is most frustrating about this scenario and how do you think do you would deal with it?

Nobody has responded yet.

Hang tight! Responses are coming in.

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What is most frustrating about this scenario and how do you think do you would deal with it?

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Communication Strategies to Redirect an Emotionally Charged Clinical Encounter

Table 4. Communication Strategies to Redirect an Emotionally Charged Clinical Encounter

<i>Strategy</i>	<i>Physician actions</i>	<i>Examples</i>
Active listening	Understand the patient's priorities, let the patient talk without interruption, recognize that anger is usually a secondary emotion (e.g., to abandonment, disrespect)	"Please explain to me the issues that are important to you right now." "Help me to understand why this upsets you so much."
Validate the emotion and empathize with the patient (understanding, not necessarily sharing, the emotion with the patient)	Name the emotion; if you are wrong, the patient will correct you; disarm the intense emotion by agreement, if appropriate	"I can see that you are angry." "You are right—it's annoying to sit and wait in a cold room." "It sounds like you are telling me that you are scared."
Explore alternative solutions	Engage the patient to find specific ways to handle the situation differently in the future	"If we had told you that appointments were running late, would you have liked a choice to wait or reschedule?" "What else can I do to help meet your expectations for this visit?" "Is there something else you need to tell me so that I can help you?"
Provide closure	Mutually agree on a plan for subsequent visits to avoid future difficulties	"I prefer to give significant news in person. Would you like early morning appointments so you can be the first patient of the day?" "Would you prefer to be referred to a specialist, or to follow up with me to continue to work on this problem?"

Cannarella Lorenzetti R, Jacques CH, Donovan C, Cottrell S, Buck J. Managing difficult encounters: understanding physician, patient, and situational factors. Am Fam Physician. 2013 Mar 15;87(6):419-25. PMID: 23547575.

What to avoid:

- ❖ inappropriate choice of words and phrases, perhaps due to assumptions being made about the patient's level of health literacy or understanding of human biology;
- ❖ lack of planned structure in delivering difficult news (e.g. scattered information confusing patients or no clear plan at all);
- ❖ inappropriate choice of setting to deliver difficult news;
- ❖ lack of options offered to the patient;
- ❖ not involving the patient in the decision-making process (e.g. treatment decisions taken without involving them and without addressing their needs and wishes);
- ❖ rushing the consultation due to other pressures
- ❖ not referring the patient to appropriate support services/resources (e.g. counselling, palliative care, support groups and quality trusted information).

<https://pmc.ncbi.nlm.nih.gov/articles/PMC5467659/>

Learn from those who do it well

- Collaboration
(vs. opposition)
- Appropriate use of power
(vs. misuse of power or violation of boundaries by either party)
- Empathy
(vs. compassion fatigue)

Table 3. Management and Coping Strategies Given by Physician Participants (N = 101)*

Encounter Component	Management and Coping Strategy	Examples of Strategy	Frequency of Mentions
Collaboration	Priority setting	Prioritize patient concerns	20
		Thorough history, physical and testing	13
		Explain fully	13
	Diagnostic skills	Be consistent and objective	12
		Facilitate patient decision making	12
		Be honest and fair	11
	Decision making	Use referrals (mental health, pain, etc)	12
		Enlist/see family	2
		Provide quality care	1
	Coaching	Set small, achievable goals	8
Appropriate use of power	Set clinical management rules	Short term symptom relief	2
		Schedule patient frequently, longer visits	26
		Clinic time management	1
	Set boundaries and limits	Good documentation	1
		Allow a tincture of time	1
		Make explicit rules	19
	Empathy	Limit number of patient concerns	10
		Set general limits	9
		Limit time at each visit	4
	Consistently unsuccessful patient encounter	Empathy	Understand patients psyche and emotions
Be compassionate and firm			8
Be patient centered			1
Reinforce positives			1
Keep professional distance			1
Opposition, misuse of power, compassion fatigue	Termination	Dismiss patient, make them want to leave	11
		Ignore problems	1
		Charge more	1

* Participants could mention more than one strategy.

2006 study of 102 physicians who were recommended by medical school faculty for excelling at difficult encounters. Interviewed on what strategies they employed that helped.
<https://www.jabfm.org/content/jabfp/19/6/533.full.pdf>

On Campus Difficult Encounters Lab

- ◇ 4 SP encounters per group
- ◇ One student per SP encounter
- ◇ 7-8 minutes per patient
- ◇ 7-8 minutes for reflection and feedback
- ◇ Professional Attire – Business Casual and White coat

LAB Assignment

Your participation in this lab is all that is required – your faculty will document this on the attendance sheet.